

PPOM1: Neuropsychology and Psychological Assessment

Lisa Hoffman, Ph.D.
Associate Professor
Dept. of Family Medicine
Lisa.Hoffman@nyit.edu

Session Objectives

THE STUDENT PHYSICIANS WILL BE ABLE TO:

1. Understand and become familiar with common psychological tests and identify their usefulness in patient populations.
2. Comprehend the differences between Objective and Projective tests
3. Comprehend the utility of Neuropsychological Examinations

Overview of Diagnostic Rating Scales and Psychiatric Instruments

Uses of Diagnostic Psychiatric Instruments:

- To help ensure the accuracy of a diagnosis (supplementing clinical interview, review of medical records and collateral sources) for clinical and research purposes
- To quantify the severity of symptoms
- To quantify the effectiveness, or lack thereof, of a given treatment modality

- **USES:**
- Diagnostic Interviews
- Helps increase reliability of psychiatric diagnoses (due to perceived unreliability; from different countries or cultural backgrounds)
- Example: Structured Clinical Interview for DSM-V (SCID) The most commonly used diagnostic instrument in psychiatry

- **Structured Clinical Interview for DSM-V (SCID)**
- A semi-structured interview that applies DSM-V criteria to a patient that covers most of the major disorders
- Administered by a clinician and may take upwards of 2 hours to complete
- Other example: Mini-International Neuropsychiatric Interview (MINI) Administered by a clinician and takes only 15-30 minutes; yes/no

Psychological Assessment

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Overview:

A psychological test is a measurement tool made up of a series of standard stimuli (i.e., questions or visual stimuli) which are administered following a standard format under a standard set of instructions which are then scored and interpreted against a normative sample allowing for an accurate evaluation of the patient's performance

Depression Scales

Example: Hamilton Rating Scale for Depression

- *(HAM-D) Aims to quantify the severity of depression in patients who already have a diagnosis of major depression
- *Administered by a clinician and takes approximately 20 minutes-used to create tx plans
- *A useful tool for measuring the progress of a patient during the course of treatment, in either research or the clinical setting
- *Is the most widely studied instrument for depression, and its reliability and validity are high
- *Scores on HAM-D-17: 0-7 (not depressed), 7-15 (mildly depressed), 15-25 (moderately depressed), over 25 (severely depressed)

EXPRESSED MOOD

Moody attitude, pessimism about the future, feeling of sadness, tendency to weep).
Absent.
Sadness, etc.
Occasional weeping.
Frequent weeping.
Extreme symptoms.

FEELINGS OF GUILT

Absent.
Self-reproach, feels he/she has let people down.
Ideas of guilt.
Present illness is a punishment; delusions of guilt.
Hallucinations of guilt.

SUICIDE

Absent.
Feels life is not worth living.
Wishes he/she were dead.
Suicidal ideas or gestures.
Attempts at suicide.

SOMNIA - Initial

Difficulty in falling asleep).
Absent.
Occasional.
Frequent.

SOMNIA - Middle

Complaints of being restless and disturbed during the night. Waking during the night).
Absent.
Occasional.
Frequent.

6. INSOMNIA - Delayed

(Waking in early hours of the morning and unable to fall asleep again).
0 = Absent.
1 = Occasional.
2 = Frequent.

7. WORK AND INTERESTS

0 = No difficulty.
1 = Feelings of incapacity, listlessness, indecision and vacillation.
2 = Loss of interest in hobbies, decreased social activities.
3 = Productivity decreased.
4 = Unable to work. Stopped working because of present illness only. (Absence from work after treatment or recovery may rate a lower score).

8. RETARDATION

(Slowness of thought, speech, and activity; apathy; stupor).
0 = Absent.
1 = Slight retardation at interview.
2 = Obvious retardation at interview.
3 = Interview difficult.
4 = Complete stupor.

9. AGITATION

(Restlessness associated with anxiety).
0 = Absent.
1 = Occasional.
2 = Frequent.

10. ANXIETY - PSYCHIC

0 = No difficulty.
1 = Tension and irritability.
2 = Worrying about minor matters.
3 = Apprehensive attitude.
4 = Fears.

11. ANXIETY - SOMATIC

Gastrointestinal, indigestion Cardiovascular, palpitation, Headaches Respiratory, Genitourinary, etc.
0 = Absent.
1 = Mild.
2 = Moderate.
3 = Severe.
4 = Incapacitating.

12. SOMATIC SYMPTOMS

GASTROINTESTINAL

(Loss of appetite, heavy feeling in abdomen; constipation).
0 = Absent.
1 = Mild.
2 = Severe.

13. SOMATIC SYMPTOMS

GENERAL

(Heaviness in limbs, back or head; diffuse backache; loss of energy and fatigability).
0 = Absent.
1 = Mild.
2 = Severe.

14. GENITAL SYMPTOMS

(Loss of libido, menstrual disturbances).
0 = Absent.
1 = Mild.
2 = Severe.

15. HYPOCHONDRIASIS

0 = Not present.
1 = Self-absorption (bodily).
2 = Preoccupation with health.
3 = Querulous attitude.
4 = Hypochondriacal delusions.

16. WEIGHT LOSS

0 = No weight loss.
1 = Slight.
2 = Obvious or severe.

17. INSIGHT

(Insight must be interpreted in terms of patient's understanding and background).
0 = No loss.
1 = Partial or doubtful loss.
2 = Loss of insight.

TOTAL ITEMS 1 TO 17: 17

0 - 7 = Normal

8 - 16 = mild depression

17 - 23 = Moderate Depression

≥ 24 = Severe Depression

Example: Beck Depression Inventory (BDI);

The BDI-II is a self-report measure comprised of 21 items reflecting specific cognitive, affective, and physical symptoms of depression. Scores range from 0 to 3, with higher numbers indicating greater symptom severity. If more than one statement from a given item is chosen by the patient, the statement of greatest severity is scored. The maximum total score is 63

- * Most widely used to assess depressive symptoms in adolescents and adults

- * Self-administered; 5-10 minutes

- * May be used as a screening tool or to determine the degree of improvement over time

Beck's Depression Inventory

This depression inventory can be self-scored. The scoring scale is at the end of the questionnaire.

1.

0 I do not feel sad.

1 I feel sad

2 I am sad all the time and I can't snap out of it.

3 I am so sad and unhappy that I can't stand it.

2.

0 I am not particularly discouraged about the future.

1 I feel discouraged about the future.

2 I feel I have nothing to look forward to.

3 I feel the future is hopeless and that things cannot improve.

3.

0 I do not feel like a failure.

1 I feel I have failed more than the average person.

2 As I look back on my life, all I can see is a lot of failures.

3 I feel I am a complete failure as a person.

4.

0 I get as much satisfaction out of things as I used to.

1 I don't enjoy things the way I used to.

2 I don't get real satisfaction out of anything anymore.

3 I am dissatisfied or bored with everything.

5.

0 I don't feel particularly guilty

1 I feel guilty a good part of the time.

2 I feel quite guilty most of the time.

3 I feel guilty all of the time.

6.

0 I don't feel I am being punished.

1 I feel I may be punished.

2 I expect to be punished.

3 I feel I am being punished.

7.

0 I don't feel disappointed in myself.

1 I am disappointed in myself.

2 I am disgusted with myself.

3 I hate myself.

8.

0 I don't feel I am any worse than anybody else.

1 I am critical of myself for my weaknesses or mistakes.

2 I blame myself all the time for my faults.

3 I blame myself for everything bad that happens.

9 0 I don't have any thoughts of killing myself.

1 I have thoughts of killing myself, but I would not carry them out.

2 I would like to kill myself.

3 I would kill myself if I had the chance.

10 0 I don't cry any more than usual.

1 I cry more now than I used to .

2 I cry all the time now.

3 I used to be able to cry , but now I can't cry even though I want to.

11 0 I am no more irritated by things that I ever was.

1 I am slightly more irritated now than usual.

2 I am quite annoyed or irritated a good deal of the time.

3 I feel irritated all the time

12.

0 I have not lost interest in other people.

1 I am less interested in other people than I used to be.

2 I have lost most of my interest in other people.

3 I have lost all of my interest in other people.

13.

0 I make decisions about as well as I ever could.

1 I put off making decisions more than I used to.

2 I have greater difficulty in making decisions more than I used to.

3 I can't make decisions at all anymore.

14. 0 I don't feel that I look any worse than I used to.

1 I am worried that I am looking old or unattractive.

2 I feel there are permanent changes in my appearance that make me look unattractive

3 I believe that I look ugly.

15.

0 I can work about as well as before.

1 It takes an extra effort to get started at doing something.

2 I have to push myself very hard to do anything.

3 I can't do any work at all.

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- 16.
- 0 I can sleep as well as usual.
 - 1 I don't sleep as well as I used to.
 - 2 I wake up 1-2 hours earlier than usual and find it hard to get back to sleep.
 - 3 I wake up several hours earlier than I used to and cannot get back to sleep.
- 17.
- 0 I don't get more tired than usual.
 - 1 I get tired more easily than I used to.
 - 2 I get tired from doing almost anything.
 - 3 I am too tired to do anything.
- 18.
- 0 My appetite is no worse than usual.
 - 1 My appetite is not as good as it used to be.
 - 2 My appetite is much worse now.
 - 3 I have no appetite at all anymore.

19.

0 I haven't lost much weight, if any, lately.

1 I have lost more than five pounds.

2 I have lost more than ten pounds.

20 0 I am no more worried about my health than usual.

I am worried about physical problems like aches, pain, upset stomach or constipation.

2 I am very worried about physical problems and it's hard to think of much else.

3 I am so worried about my physical problems that I cannot think of anything else.

21 0 I have not noticed any recent changes in my interest in sex.

1 I am less interested in sex than I used to be.

2 I have almost no interest in sex.

3 I have lost interest in sex completely.

INTERPRETING THE BECK DEPRESSION INVENTORY

*Now that you have completed the questionnaire, add up the score for each of the twenty-one questions by counting the number to the right of each question you marked. The highest possible score is 63 and the lowest is zero. You can evaluate your depression according to the table below.

TOTAL SCORE _____ 1-10 These ups and downs are considered normal. 11-16 Mild mood disturbances. 17-20 Borderline clinical depression. 21-30 Moderate depression. 31-40 Severe depression. over 40 Extreme depression.

Psychosis Scales

- **Brief Psychiatric Rating Scale** (BPRS) Covers symptoms of psychosis, but also addresses depression, and anxiety symptoms-18 symptom constructs
- -useful in gauging efficacy of treatment in pts with psychoses

* **Positive and Negative Symptoms Scale**
(PANSS) Frequently used in trials of antipsychotic drugs

NAME: _____
 PATIENT ID#: _____

DATE: _____
 MD: _____

BRIEF PSYCHIATRIC RATING SCALE (BPRS)

Please enter the score for the term which best describes the patient's condition.

0 = not assessed, 1 = not present, 2 = very mild, 3 = mild, 4 = moderate, 5 = moderately severe, 6 = severe, 7 = extremely severe

1. SOMATIC CONCERN Degree of concern over present bodily health. Rate the degree to which physical health is perceived as a problem by the patient, whether complaints have a realistic basis or not. SCORE	10. HOSTILITY Animosity, contempt, belligerence, disdain for other people outside the interview situation. Rate solely on the basis of the verbal report of feelings and actions of the patient toward others; do not infer hostility from neurotic defenses, anxiety, nor somatic complaints. (Rate attitude toward interviewer under "uncooperativeness"). SCORE
2. ANXIETY Worry, fear, or over-concern for present or future. Rate solely on the basis of verbal report of patient's own subjective experiences. Do not infer anxiety from physical signs or from neurotic defense mechanisms. SCORE	11. SUSPICIOUSNESS Brief (delusional or otherwise) that others have now, or have had in the past, malicious or discriminatory intent toward the patient. On the basis of verbal report, rate only those suspicions which are currently held whether they concern past or present circumstances. SCORE
3. EMOTIONAL WITHDRAWAL Deficiency in relating to the interviewer and to the interview situation. Rate only the degree to which the patient gives the impression of failing to be in emotional contact with other people in the interview situation. SCORE	12. HALLUCINATORY BEHAVIOR Perceptions without normal external stimulus correspondence. Rate only those experiences which are reported to have occurred within the last week and which are described as distinctly different from the thought and imagery processes of normal people. SCORE
4. CONCEPTUAL DISORGANIZATION Degree to which the thought processes are confused, disconnected, or disorganized. Rate on the basis of integration of the verbal products of the patient; do not rate on the basis of patient's subjective impression of his own level of functioning. SCORE	13. MOTOR RETARDATION Reduction in energy level evidenced in slowed movements. Rate on the basis of observed behavior of the patient only; do not rate on the basis of patient's subjective impression of own energy level. SCORE
5. GUILT FEELINGS Over-concern or remorse for past behavior. Rate on the basis of the patient's subjective experiences of guilt as evidenced by verbal report with appropriate affect; do not infer guilt feelings from depression, anxiety or neurotic defenses. SCORE	14. UNCOOPERATIVENESS Evidence of resistance, unfriendliness, resentment, and lack of readiness to cooperate with the interviewer. Rate only on the basis of the patient's attitude and responses to the interviewer and the interview situation; do not rate on basis of reported resentment or uncooperativeness outside the interview situation. SCORE
6. TENSION Physical and motor manifestations of tension "nervousness", and heightened activation level. Tension should be rated solely on the basis of physical signs and motor behavior and not on the basis of subjective experiences of tension reported by the patient. SCORE	15. UNUSUAL THOUGHT CONTENT Unusual, odd, strange or bizarre thought content. Rate here the degree of unusualness, not the degree of disorganization of thought processes. SCORE
7. MANNERISMS AND POSTURING Unusual and unnatural motor behavior, the type of motor behavior which causes certain mental patients to stand out in a crowd of normal people. Rate only abnormality of movements; do not rate simple heightened motor activity here. SCORE	16. BLUNTED AFFECT Reduced emotional tone, apparent lack of normal feeling or involvement. SCORE
8. GRANDIOSITY Exaggerated self-opinion, conviction of unusual ability or powers. Rate only on the basis of patient's statements about himself or self-in-relation-to-others, not on the basis of his demeanor in the interview situation. SCORE	17. EXCITEMENT Heightened emotional tone, agitation, increased reactivity. SCORE
9. DEPRESSIVE MOOD Despondency in mood, sadness. Rate only degree of despondency; do not rate on the basis of inferences concerning depression based upon general retardation and somatic complaints. SCORE	18. DISORIENTATION Confusion or lack of proper association for person, place or time. SCORE

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Anxiety Scales

- **Hamilton Rating Scale for Anxiety** (HAM-A) The most widely used scale for measurement of anxiety
- **Yale-Brown Obsessive Compulsive Scale** (Y-BOCS) The most widely used scale for assessment of severity of obsessive-compulsive disorder symptoms
- GAD-7 and PHQ-9-used in ECW at NYITCOM-brief screenings of anxiety and depression

Cognitive Impairment Scales

Overview:

- *Are useful as an initial screen for organic psychopathology
- *Results of these scales can help the clinician decide whether or not to request formal neuropsychological testing or imaging studies
- *May be influenced by the patient's intelligence, level of education, and literacy

Cognitive Impairment Scales

Examples: *Folstein Mini-Mental State Examination
(MMSE)

*The most widely used instrument for measuring cognitive impairment

*May be administered by non-clinicians; takes less than 10 minutes

Folstein Mini-Mental State Examination

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Screening Tool: The Mini-Mental State Examination (MMSE)

Patient _____ Examiner _____ Date _____

Maximum	Score	
		Orientation
5		• What is the (year) (season) (date) (day) (month)?
5		• Where are we (state) (country) (town) (hospital) (floor)?
		Registration
3		• Name 3 objects: 1 second to say each. Then ask the patient all 3 after you have said them. Give 1 point for each correct answer. Then repeat until he/she learns all 3. Count trials and record. Trials _____
		Attention and Calculation
5		• Serial 7's. 1 point for each correct answer. Stop after 5 answers. Alternatively spell "world" backward.
		Recall
3		• Ask for the 3 objects repeated above. Give 1 point for each correct answer.
		Language
2		• Name a pencil and watch.
1		• Repeat the following "No ifs, ands or buts."
3		• Follow a 3-stage command: "Take a paper in your hand, fold it in half and put it on the floor."
1		• Read and obey the following CLOSE YOUR EYES.
1		• Write a sentence.
1		• Copy the design shown.
		

_____ **Total Score**

ASSESS level of consciousness along a continuum _____

Alert Drowsy Stupor Coma

"Mini-Mental State." A Practical Method for Grading the Cognitive State of Patients for the Clinician. *Journal of Psychiatric Research*, 12(3): 189-198, 1975.
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more information on reverse →

Montreal Cognitive Assessment (MoCA)

*The Montreal Cognitive Assessment (MoCA) was designed as a rapid screening instrument for detecting Mild Cognitive Impairment (MCI)-55-85y/o. It assesses different cognitive domains: attention and concentration, executive functions, memory, language, visuoconstructional skills, conceptual thinking calculations, and orientation. Time to administer the MoCA is approximately 10 minutes. The total possible score is 30 points; a score of 26 or above is considered normal.

Montreal Cognitive Assessment

MONTREAL COGNITIVE ASSESSMENT (MOCA)
Version 7.1 Original Version

NAME :

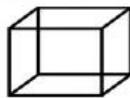
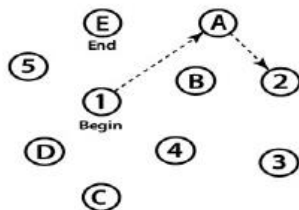
Education :

Sex :

Date of birth :

DATE :

VISUOSPATIAL / EXECUTIVE



Copy
cube

Draw CLOCK (Ten past eleven)
(3 points)

POINTS

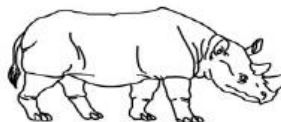
[] Contour [] Numbers [] Hands

___/5

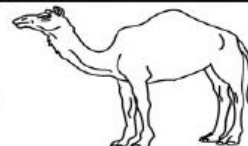
NAMING



[]



[]



[]

___/3

MEMORY

Read list of words, subject must repeat them. Do 2 trials, even if 1st trial is successful. Do a recall after 5 minutes.

FACE VELVET CHURCH DAISY RED

1st trial

2nd trial

No points

ATTENTION

Read list of digits (1 digit/ sec.).

Subject has to repeat them in the forward order

[] 2 1 8 5 4

Subject has to repeat them in the backward order

[] 7 4 2

___/2

Read list of letters. The subject must tap with his hand at each letter A. No points if ≥ 2 errors

[] FBACMNAAJKLBAFAKDEAAAJAMOF AAB

___/1

Serial 7 subtraction starting at 100

[] 93

[] 86

[] 79

[] 72

[] 65

4 or 5 correct subtractions: **3 pts.** 2 or 3 correct: **2 pts.** 1 correct: **1 pt.** 0 correct: **0 pt**

___/3

LANGUAGE

Repeat : I only know that John is the one to help today. []

The cat always hid under the couch when dogs were in the room. []

___/2

Fluency / Name maximum number of words in one minute that begin with the letter F:

[] _____ (N ≥ 11 words)

___/1

ABSTRACTION

Similarity between e.g. banana - orange = fruit

[]

train - bicycle

[]

watch - ruler

[]

___/2

DELAYED RECALL

Has to recall words

WITH NO CUE

[]

FACE

[]

VELVET

[]

CHURCH

[]

DAISY

[]

RED

[]

Points for

UNCUED

recall only

___/5

Optional

Category cue

Multiple choice cue

ORIENTATION

[] Date

[] Month

[] Year

[] Day

[] Place

[] City

___/6

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Normal ≥ 26 / 30

TOTAL

Add 1 point if ≤ 12 yr edu

___/30

Administered by: _____

Intelligence Tests

IQ Testing Example: Weschler series (preschool, children, adults) Provides three major IQ test scores: the Full Scale IQ, the Verbal IQ (vocabulary, similarities, arithmetic, digit span, information, and comprehension), and the Performance IQ (picture completion, digit symbol, block design, matrix reasoning, and picture arrangement)

All have mean of 100

Full-Scale IQ >130-very superior, 120-129-superior, 110-119-high average, 90-109-average, 80-89-low average, 70-79-borderline, <69-mentally retarded

Tests of Personality, Psychopathology and Psychological Functioning

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Objective Tests: Also known as self-report tests, are designed to clarify and quantify a patient's general personality functioning and psychopathology

Example: Minnesota Multiphasic Personality Inventory-2 (MMPI-2)

- A 567-item true/false self-report test of psychological functioning

- Contains ten clinical scales that assess major categories of psychopathology

- Four scales of validity are used to assess for the accuracy and truthfulness of the test-takers answers

Similar: Millon-Clinical Multiaxial Inventory-III (MCMI-III)

Questions from Minnesota Multiphasic Personality Inventory-II

- ☐ False ☐ True 559. The people I work with are not sympathetic with my problems.
- ☐ False ☐ True 560. I am satisfied with the amount of money I make.
- ☐ False ☐ True 561. I usually have enough energy to do my work.
- ☐ False ☐ True 562. It is hard for me to accept compliments.
- ☐ False ☐ True 563. In most marriages one or both partners are unhappy.
- ☐ False ☐ True 564. I almost never lose self-control.
- ☐ False ☐ True 565. It takes a great deal of effort for me to remember what people tell me these days.
- ☐ False ☐ True 566. When I am sad or blue, it is my work that suffers.
- ☐ False ☐ True 567. Most married couples don't show much affection for each other.

Score

Tests of Personality, Psychopathology and Psychological Functioning

Projective Tests:

- Differ from objective tests in that they are less structured and require more effort on the part of the patient to make sense of and to respond to, the test stimuli
- Provide insights into the patient's typical style of perceiving, organizing, and responding to ambiguous external and internal stimuli

Projective Tests:

Example: Rorschach Inkblot Test

- test of whole personality functioning
- Consists of 10 cards of symmetrical inkblots; “What might this be?”
- Particularly useful for quantifying a patient’s reality contact and the quality of their thinking
- Requires the subject to interpret the questions by projecting his or her emotions, conflicts and defense mechanisms into the answers
- Examiner must have special training in interpretation of subject’s responses
- Most commonly used projective personality test

Rorschach

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Example: **Thematic Apperception Test (TAT)**

- Useful in revealing a patient's dominant motivations, emotions, and core personality conflicts
- Consists of a series of 20 cards depicting people in various interpersonal interactions; ambiguous scenes; "Make up a story around this picture"

Example: **Sentence Completion Test**

- Used to identify a patient's worries and motivations by verbal associations
- Believed to reveal a patient's underlying thoughts or feelings
- Patient is given a sentence stem and then they complete the rest of the sentence

Thematic Apperception Test

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Neuropsychological Assessment

Overview:

The main goal of a neuropsychological evaluation is to:

- Better understand the relationship between your brain and behavior, mood and thinking (cognitions).

- Such tests can help with Diagnoses, pinpoint cognitive strengths and deficits, aid with treatment planning.

Neuropsychological Assessment

Areas of assessment:

- Attention and concentration
- Receptive and expressive language
- Memory functioning
- Visual-spatial constructional ability
- Executive functions and abstract thinking
- Motor and sensory functioning
- Emotional function
- Processing Speed

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Examples of Neuropsychological Assessments:

- Halstead-Reitan battery of neuropsychological tests
- Brief Neuropsychological Test example:
Dementia Rating Scale (DRS)
- Repeatable Battery of Adult Neuropsychological
Status (RBANS)

Dementia Rating Scale (DRS)-2

-Assesses cognitive function in those with suspected dementia.

-Five Domains: 1-Attention

2-Initiation/Perseveration

3-Construction

4-Conceptualization

5- Memory

-Score-0-144 higher score better cognition-No impairment, Mild, Moderate, Severe Impairment

Repeatable Battery for Neuropsychological Status (RBANS)

- Brief neuropsychological screening battery widely used to assess cognitive function in dementia, head injury.
- used in 20-89 y/o, research protocols-easy/quick, alternate forms
- 12 Subtests: Immediate and Delayed Memory, Attention, Visuospatial/Construction, Language
- Index Scores: Very Superior, Superior, High Average, Average, Low Average, Borderline & Extremely Low

Summary & References

Summary Slide

- Understand common psychological tests and identify their usefulness in patient populations
- Be knowledgeable about common neuropsychological tests and their utility in patient populations

References

(*Slides adapted from Dr. Kalash's Lecture)

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- Psychiatric Times-3/19/2-
BPRS<https://www.psychiatrictimes.com/view/bprs-brief-psychiatric-rating-scale>

References

(*Slides adapted from Dr. Kalash's Lecture)

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[APA - The Structured Clinical Interview for DSM-5® \(appi.org\)](#)

The Repeatable Battery for the Assessment of Neuropsychological Status Effort Scale

[Get access](#)

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- Questions?
- Lisa.Hoffman@nyit.edu
- Available for Virtual Office Hours

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